



PREVENTION, IDENTIFICATION AND MANAGEMENT OF MATERNAL INFECTION



Topic 9



PREVENTION



INTRAPARTUM



POSTNATAL



1. ANTENATAL INFECTION PREVENTION & EARLY DETECTION



ASYMPTOMATIC BACTERIURIA (ASB)

- Screen by urinalysis (leukocyte esterase, nitrite) or urine culture (if available)
- Treat if positive to prevent pyelonephritis, preterm labour, low birth weight



HIV

- Offer HIV test to all pregnant women
- Counsel, test & link to ART and care



SYPHILIS

- Screen using RPR/VDRL (or rapid test)
- Treat with Benzathine Penicillin to prevent congenital syphilis



HEPATITIS B

- Screen HBsAg
- If positive – counsel, refer & manage



MALARIA

- RDT for malaria at every ANC visit
- Treat as per national guidelines



OTHER IMPORTANT SCREENING

- Hemoglobin – every visit
- Multi-reagent uristix – every visit (Albumin, Sugar, ASB – Leukocyte & Nitrate)
- OGTT – at 1st visit & 24–28 weeks (3rd visit)

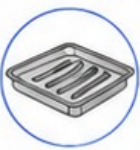


INFECTION PREVENTION IN PREGNANCY

- Hand hygiene, safe food and drinking water
- Personal hygiene – daily bath, clean clothes
- Avoid contact with sick persons
- Regular ANC visits and follow medical advice
- Complete recommended vaccinations (e.g., Td)

2. INTRAPARTUM INFECTION PREVENTION

UNIVERSAL PRECAUTIONS FOR EVERY BIRTH

	HAND HYGIENE	• Hand hygiene before and after every procedure
	USE OF GLOVES	• Use of gloves for every vaginal examination and delivery
	STERILE OR HIGH-LEVEL DISINFECTED INSTRUMENTS AND EQUIPMENT	• Use sterile or high-level disinfected instruments and equipment
	SAFE INJECTION PRACTICES – USE STERILE DISPOSABLE SYRINGES & NEEDLES	• Safe injection practices – use sterile disposable syringes & needles
	SAFE DISPOSAL OF SHARPS AND BIOMEDICAL WASTE	• Safe disposal of sharps and biomedical waste
	CLEAN DELIVERY AREA, LINEN AND SURFACES	• Clean delivery area, linen and surfaces
	LIMIT VAGINAL EXAMINATIONS	• Limit Vaginal examination • Use sterile swab and antiseptic solution for perineal clean • Don't perform more than 4 PV through out labour (minimum 4 hour gap should be ensure)
	ONE BIRTH COMPANION AT A TIME	• One birth companion at a time

3. POSTPARTUM INFECTION PREVENTION

GOOD PRACTICES AFTER DELIVERY



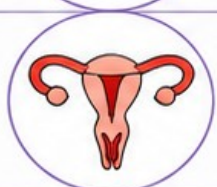
HAND HYGIENE

- Hand hygiene before and after caring for the mother or newborn



PERINEAL CARE

- Keep perineum clean and dry. Change pads regularly



MONITOR LOCHIA

- Monitor lochia – report foul-smelling discharge, fever or pain early



EARLY INITIATION OF BREASTFEEDING

- Encourage early initiation of breastfeeding



NUTRITIOUS DIET, REST AND ADEQUATE FLUIDS

- Nutritious diet, rest and adequate fluids



RECOGNIZE DANGER SIGNS EARLY AND SEEK CARE

- Recognize danger signs early and seek care

RISK FACTORS

CLIENT-RELATED (MATERNAL)



Pre-existing STIs/RTIS



Not immunized against tetanus and Diphtheria



Pre-existing diabetes



Poor patient hygiene



Pre-existing anaemia and malnutrition

RELATED TO DELIVERY



Failure to follow Infection prevention practices



Poor monitoring of mother during and after labour



Frequent vaginal examinations



PROM >18 hrs



Prolonged/obstructed labour >24 hrs



C-section/ Instrumental delivery



Manipulations high in the birth canal



Dead tissue in the birth canal

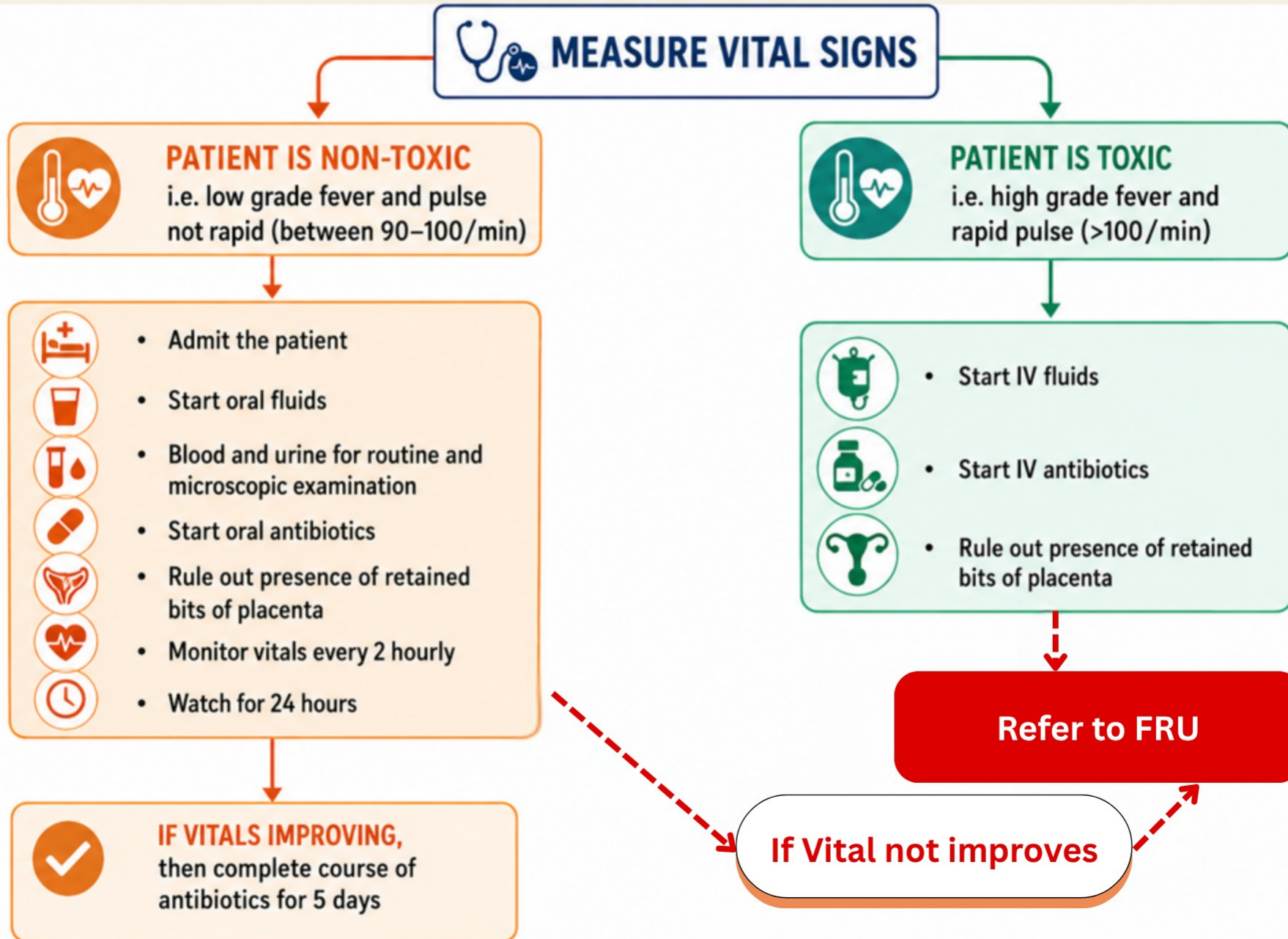


Unrepaired vaginal/cervical lacerations



PPH

PRINCIPLES OF MANAGEMENT OF PUERPERAL SEPSIS



Recommended Antibiotic to Mother

Not very Sick Mother	Very Sick Mother
Ampicillin 1 gm - 6 hourly oral/ Amoxicillin/ Ceftriaxone + Sulbactam Metronidazole 400mg - 8 hourly oral Gentamycin 80 mg, BD Intra Muscularly	Piperacillin/Tazobactam 4.5g IV q6h Metronidazole 400mg iv 8 hourly Amikacin 15mg/kg body wt iv

As per WHO guidelines- Clinical response evidence based antibiotics should be administered.

Key messages

- Puerperal sepsis is one of the leading causes of maternal deaths
- Puerperal sepsis is to a great extent preventable
- Antibiotics should be given to the woman only if they are indicated
- At periphery, ANM should identify the indications for antibiotics and give first dose before referral